

1. Administrative & Study Identification

Full Study Title: A Phase III, Multicenter, Double-Blind, Placebo-Controlled Study to Assess the Efficacy and Safety of Induction Therapy With R07790121 in Patients With Moderately to Severely Active Crohn's Disease **Short Title/Acronym:** SIBERITE-2 **Protocol Number:** GA45332 **EU Trial (CTIS) Number:** 2024-513054-30-00 **NCT Number:** NCT06819891 **Overall Status:** Recruiting **Sponsor Name:** Hoffmann-La Roche **Investigational Product:** Afimkibart (R07790121 / TL1A inhibitor) **Phase of Development:** Phase 3 **Medical Monitor:** Genentech / Hoffmann-La Roche Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the efficacy and safety of induction therapy with Afimkibart (R07790121) compared to placebo in participants with moderately to severely active Crohn's disease (CD). **Secondary Objectives:** To assess the pharmacokinetics, endoscopic response, clinical remission rates, and overall quality of life improvements, as well as to define the safety and tolerability profile of Afimkibart during the induction period.

2.2 Background & Rationale Crohn's disease (CD) causes chronic inflammation of the digestive tract. Standard treatments for CD include corticosteroids, biologics, and small molecules; however, many patients do not experience symptom improvement or they lose response over time. Afimkibart (R07790121) is a novel investigational fully human IgG1 mAb against TL1A that specifically targets pathways involved in the inflammatory process, potentially reducing side effects and improving efficacy. This study provides critical Phase III data to determine if Afimkibart's targeted induction approach can address the unmet clinical need for patients who have failed conventional or advanced CD therapies.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Primary Purpose:** Treatment **Control Type:** Placebo-controlled **Intervention Model:** Parallel Assignment **Blinding (Masking):** Double-blind (Quadruple Masking: Participant, Care Provider, Investigator, Outcomes Assessor) **Randomization (Allocation):** Randomized (1:1 or similarly proportioned to Afimkibart or Placebo)

3.2 Planned Enrollment & Duration Target **\$N\$**: Approximately 425 participants **Number of Sites**: 162 locations globally (including the US, Europe, and China) **Study Start (Actual)**: 12-May-2025 **Estimated Primary Completion**: 31-Dec-2028 **Estimated Study Completion**: 30-Apr-2033

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. All genders (Male/Female), 16 to 80 years of age. 2. Confirmed diagnosis of moderately to severely active Crohn's disease via endoscopy. 3. Bodyweight ≥ 40 kg. 4. Demonstrated inadequate response, loss of response, and/or intolerance to at least one protocol-specified conventional or advanced CD therapy.

4.2 Exclusion Criteria 1. Diagnosis of short gut or short bowel syndrome, symptomatic bowel strictures, fulminant colitis, toxic megacolon, or presence of an ileostomy, colostomy, or ileoanal pouch. 2. Current diagnosis of ulcerative colitis (UC) or indeterminate colitis, ischemic colitis, infectious colitis, radiation colitis, or microscopic colitis. 3. History of ≥ 3 bowel resections. 4. Presence of abdominal or perianal abscess, or severe fistulas (e.g., rectovaginal, enterovesical, high output enterocutaneous, or perianal fistulas with >3 openings). 5. Pregnancy, breastfeeding, or intention to become pregnant. Males and females of childbearing potential must meet protocol criteria for contraception requirements. 6. Active infections including C. difficile, CMV, HIV, Hepatitis B (HBV), Hepatitis C (HCV), or Tuberculosis (TB) (active, latent not successfully treated, or inadequately treated) during screening. 7. Past or current evidence of GI cancer, definite low-grade or high-grade colonic dysplasia, or non-gastrointestinal cancer (except adequately treated non-metastatic basal/squamous cell skin cancer or in situ cervical cancer). 8. Has received protocol-specified prohibited medicines, including known exposure to any type of anti-TL1A therapy.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Induction therapy utilizing a dual delivery method. **Route of Administration**: Intravenous (IV) infusion followed by Subcutaneous (SC) injection. **Duration of Treatment**: Induction period, followed by an optional treatment extension period and a safety follow-up period. **Intervention Groups**: - Drug: Afimkibart (R07790121) - Drug: Placebo

5.2 Concomitant & Prohibited Therapy Permitted: Participants may continue to take their usual anti-inflammatory background treatments for CD as specified in the protocol. **Prohibited**:

Concurrent investigational drugs, anti-TL1A therapies, certain contraindicated immunosuppressants, and live vaccines.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to 0	Informed Consent, Endoscopy, Medical History, Infection Screening (TB, HIV, Hepatitis)
Baseline	Day 0	Randomization, First IV Infusion, Clinical Assessment
Induction	Weeks 2-12	Efficacy Assessments, PK Sampling, SC Injection transitions, Safety Labs
End of Study	Follow-up	Final Endoscopy, Exit Interview, Safety Follow-Up, Option to enter extension phase

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Efficacy of induction therapy defined by the proportion of participants achieving clinical remission and endoscopic response by the end of the induction period. **Measurement Method:** Clinical disease activity scores (e.g., CDAI) and Central Endoscopic Evaluation.

7.2 Secondary & Exploratory Endpoints 1. Change from baseline in clinical symptom severity and health-related quality of life. 2. Pharmacokinetic (PK) profile of Afimkibart (R07790121). 3. Incidence and severity of adverse events (AEs) and serious adverse events (SAEs).

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous monitoring for systemic and injection-site reactions (e.g., allergic reactions, nausea, joint pain, headaches, changes in blood pressure/temperature). **Serious Adverse Events (SAEs):** Standard reporting timelines (typically 24 hours to the sponsor). **Data Monitoring Committee (DMC):** Independent Data Monitoring Committee (iDMC) to review safety unblinded data periodically.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) population for all primary efficacy analyses; Safety Analysis Set for safety endpoints. **Sample Size Justification:** $N = 425$, sufficiently powered ($\alpha = 0.05$) to detect a statistically significant difference in clinical remission and endoscopic response between Afimkibart and placebo. **Handling of Missing Data:** Non-Responder Imputation (NRI) for categorical efficacy endpoints; mixed-model repeated measures (MMRM) for continuous endpoints.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki.
Monitoring Plan: Routine risk-based onsite and remote monitoring visits by Hoffmann-La Roche/Genentech clinical monitors. **Audit Trail:** Validated Electronic Data Capture (EDC) system with eCRF locking and comprehensive audit trails.

11. Study Identifiers & Governance

Primary ID: GA45332 **EU Trial (CTIS) Number:** 2024-513054-30-00
Registry Number: NCT06819891 **Sponsor/Lead Organization:** Hoffmann-La Roche **Study Title:** SIBERITE-2 **Official Regulatory Title:** A Phase III, Multicenter, Double-Blind, Placebo-Controlled Study to Assess the Efficacy and Safety of Induction Therapy With R07790121 in Patients With Moderately to Severely Active Crohn's Disease **Central Contact Email:** global-roche-genentech-trials@gene.com **Central Contact Phone:** 888-662-6728 (U.S. and Canada) **Patient Portal:** <https://forpatients.roche.com/>

12. Clinical Framework (Crohn's Disease Specific)

Primary Condition: Moderately to Severely Active Crohn's Disease (CD) **Diagnosis Threshold:** Confirmed via endoscopy with documented inadequate response/intolerance to ≥ 1 advanced or conventional CD therapies. **Medical Methodology:** TL1A targeted induction inhibition. **Optimization Target:** Clinical remission and intestinal mucosal healing.

13. Study Procedures & Methodology

Management Pathway: Double-blind placebo-controlled IV/SC induction **Education Protocol (HCP):** Site initiation visits covering TL1A mechanism, IV/SC administration protocols, and endoscopic grading standards. **Education Protocol (Patient):** Informed consent, background therapy continuation rules, AE symptom recognition, and contraception guidelines. **Quality Audit Mechanism:** Central reading of endoscopies and routine EDC data clarification.

14. Observation & Follow-Up Schedule

Total Duration: Active through Primary Completion (Estimated 31-Dec-2028), Study Completion (Estimated 30-Apr-2033). **Onsite Visit Cadence:** Regular intervals matching IV induction dosing and transition to SC maintenance. **Remote Monitoring Frequency:**

Routine telemedicine/symptom diary checks as dictated by the site protocol. **Checklist Review Interval:** Corresponding to primary induction efficacy milestones.

15. Sign-off & Approval

Role	Name	Signature	Date	:---	:---	:---	:---	
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]					
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]			Lead		
Statistician	[Name]	_____	[DD-MMM-YYYY]					